DSM LAB · STUDY TOGETHER

The Unfolding Patient: role-play cards

Patients 37–48.

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

Patient 37: The new knee

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Graham, 61, a high school band director, on a surgical floor two days after a knee replacement. His nurse asks for a consult, and his wife is at the bedside.

> *"I'm fine. I just need to get out of this bed and go home."*

If asked about…

When did this start?	Last night. I couldn't sleep, and then the shaking and sweating started. I feel like I'm going to jump out of my skin.
Where are you and what day is it?	I'm in the hospital. They did my left knee on Tuesday. Today is Thursday.
Are you seeing or hearing anything unusual?	No. Nothing like that. Just the beeping machines, and those are real.
Alcohol	Wine with dinner. A couple of glasses. (If pressed:) Okay, most of a bottle most nights, more on Saturdays. I've done that for years, about the same amount the whole time. My last glass was the night before surgery.
Medications and drugs	A blood pressure pill. No sleeping pills, no anxiety pills, no drugs. Here they give me pain medicine every few hours.
Medical history	High blood pressure and a bad knee. I've never had a seizure. I don't think I've gone two nights without wine in ten years, so I've never had this happen.
Worry and panic before this	I get nervous before a big concert, and then it's over. I've never had panic attacks. I usually sleep fine.
Thoughts of suicide or harming others	No. I want to get home and back to my students. I'm not going to hurt anyone.
Family history	My father drank a lot of beer, and his liver gave out in his seventies. My sister has anxiety.
Work and daily life	I teach full time and run three bands. I haven't missed a day or a rehearsal in years.
Cutting down	I've never tried. I never thought I needed to.
Culture and context	In the music world there's wine at every reception and every dinner. Nobody thinks twice about it, and I didn't think a surgeon would care.

Only if asked (don’t volunteer)

- Most of a bottle of wine every night for years
- Last drink was the night before surgery
- Told the surgeon he drinks 'socially'

Patient 37: answer and debrief

DIAGNOSIS

Alcohol withdrawal

A self-described worrier who is jumpy and sleepless after surgery looks like anxiety, and new agitation in a hospital patient raises delirium. Clue 3 argues against delirium (he is oriented and attentive), and clue 4 flips it: a nightly bottle of wine for years stopped abruptly the night before surgery, which he hid from the surgeon. Sweating, a fast pulse, tremor and insomnia that begin a day or two later fit alcohol withdrawal. Exam lesson: alcohol withdrawal is medically dangerous. Seizures typically come 24 to 48 hours after the last drink and delirium tremens 48 to 96 hours, so the medical team must know now and manage it with supervised treatment (benzodiazepines, thiamine, CIWA-Ar monitoring). Alcohol use disorder may well co-occur: withdrawal is one of its 11 criteria, and years of a nightly bottle suggest tolerance. Assess it fully once he is stable. The diagnosis that needs action now is withdrawal.

What had to be ruled out

- Delirium: oriented, attentive and not hallucinating; normal labs, oxygen and incision (watch closely, since withdrawal can progress to delirium tremens)
- Generalized anxiety disorder and panic disorder: do not explain the vital signs or the timing after his last drink; his usual worry never looked like this
- Sedative, hypnotic or anxiolytic withdrawal: takes no sleeping or anxiety pills at home
- Alcohol use disorder: may well co-occur (withdrawal is one criterion, and years of a nightly bottle suggest tolerance). Assess fully once he is stable; withdrawal is what needs action now.

Checkpoint: What would you ask or check NEXT?

A. Teach him slow breathing for his anxiety and check on him tomorrow

B. Ask him and his wife exactly how much he drinks and when he last drank, and tell the medical team now **(best)**

C. Ask the team to hold his pain medicine in case it is causing the agitation

Tremor, sweating, a fast pulse and sleeplessness one to two days into a hospital stay, with a clear mind, suggest withdrawal, which can progress to seizures and delirium tremens. Finding the substance and alerting the medical team comes first. A treats it as ordinary anxiety and loses a day. C changes his medicine without finding the cause and leaves his pain untreated.

The case in six clues (as the app reveals it)

1. Nurse: He hasn't slept since last night, he's jumpy, and he keeps asking to go home. He told me he's always been a worrier.
2. I feel like I'm about to crawl out of my skin. My heart is pounding, I'm soaked with sweat, and my hands won't hold the cup still.
3. Nurse: Pulse 116 and his blood pressure is up. But he knows where he is and the date, he can say the months backward without a slip, and he hasn't seen or heard anything odd.
4. Wife: When the surgeon asked about drinking, he said 'socially.' He has most of a bottle of wine every night and more on weekends, and has for years. His last glass was the night before surgery.
5. Nurse: His labs, oxygen and incision look fine, and there's no sign of infection. He's getting his pain medicine on schedule, and he takes no sleeping or nerve pills at home.
6. Wife: The amount hasn't crept up in ten years, he's never tried to cut back, and he's never missed work or a rehearsal over it. He worries before concerts like anyone, but I've never seen him shake like this.

What a skilled clinician weighs after each clue

1. Generalized anxiety disorder, Delirium, Alcohol withdrawal, Panic disorder
2. Panic disorder, Alcohol withdrawal, Delirium, Sedative, hypnotic or anxiolytic withdrawal
3. Alcohol withdrawal, Sedative, hypnotic or anxiolytic withdrawal, Panic disorder
4. Alcohol withdrawal, Alcohol use disorder, Delirium
5. Alcohol withdrawal, Alcohol use disorder
6. Alcohol withdrawal

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Alcohol Withdrawal; Sullivan et al. (1989), CIWA-Ar, British Journal of Addiction 84:1353-1357

Patient 38: The bus to work

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Fatima, 83, a retired hospital cook, at a nursing home where she moved two months ago because arthritis and falls made living alone unsafe. A nursing aide and the charge nurse ask for help, and her daughter arrives during the visit.

> *"Is it time for my shift? I'll miss the bus if you keep me here."*

If asked about…

When did this start?	Daughter: Three days ago. Two days before that she was beating me at cards. Then the nurse called and said she was mixed up.
Where are you and what day is it?	At the hospital kitchen? No… I don't know this room. Is it Monday? (Gets the day wrong and drifts off mid-answer.)
Sleep	Aide: She dozes most of the afternoon and is up and restless at night, trying to get out of bed.
Medicines	Nurse: A blood pressure pill, an arthritis cream and acetaminophen. Nothing new and no changes in months.
Alcohol and drugs	Daughter: She has never had a drink in her life. No sleeping pills either.
Medical history	Nurse: Arthritis, high blood pressure and two falls at home before she moved here. No strokes. The low fever started yesterday, and she's going to the bathroom often.
Mood before this week	Daughter: She was happy here. She loves the garden and the women at her table. She wasn't sad at all.
Daily life before this week	Daughter: She paid her own bills by phone banking, kept track of her pills in a weekly box, and played cards every Sunday.
Thoughts of suicide or harming others	No, no. I have to get to work. (Nurse: She is not trying to hurt herself, but she's at high risk of falling when she climbs out of bed.)
Family history	Daughter: Her older sister got forgetful in her nineties. Nobody else.
Culture and language	Daughter: She speaks Arabic and French and some English. The weekend staff only spoke English with her, so maybe they thought she wasn't making sense. She prays five times a day, and this week she's been missing prayers, which she never does.

Only if asked (don’t volunteer)

- Was sharp and managing her own bills five days ago
- Change began three days ago
- Low fever and frequent urination since yesterday
- The weekend staff spoke only English with her

Patient 38: answer and debrief

DIAGNOSIS

Delirium

Confusion in a nursing-home resident is easy to file under dementia, as the weekend staff did. Clue 3 flips it: five days ago she was sharp and managing her own bills, and the change began three days ago. A sudden onset, attention that drifts and fluctuates over the day, worse evenings, and a urinary infection point to delirium. Exam lesson: delirium is a medical emergency, so get an urgent medical evaluation and treat the cause. Cognitive problems seen only during a delirium cannot support a neurocognitive disorder diagnosis; reassess her thinking after the delirium clears.

What had to be ruled out

- Major neurocognitive disorder: sharp and independent with her bills five days ago; abrupt onset; deficits seen only during a delirium do not count
- Neurocognitive disorder with Lewy bodies: no slow course, no lasting visual hallucinations or parkinsonism
- Major depressive disorder: no sad mood before this week; poor appetite and confusion began with the infection, and attention is impaired
- Psychotic disorder: talk of the bus to work is disorientation, not a fixed false belief

Checkpoint: What would you ask or check NEXT?

A. Start planning her move to the memory unit, as the weekend staff suggested

B. Arrange full memory testing to measure how much she has declined

C. Tell the nurse and her doctor this is a sudden change that needs a medical evaluation today **(best)**

A sudden change over a few days in someone who was sharp five days ago is delirium until proven otherwise, and delirium is a medical emergency. A acts on a label without finding the cause. B delays care, and testing during an acute confusion would not show her true baseline.

The case in six clues (as the app reveals it)

1. Aide: She's been mixed up, calling me by her sister's name, and she's hardly eating. She doesn't remember that I helped her shower this morning.
2. Aide: The weekend staff wrote 'dementia, needs the memory unit' in her chart. She drifts off in the middle of a sentence and picks at her sheets.
3. Daughter: Five days ago she beat me at cards and told me every bill she'd paid this month. This started three days ago.
4. Aide: Mornings she's almost herself. By evening she doesn't know where she is, and last night she tried to climb out of bed to catch the bus to work.
5. Nurse: She has a low fever, and she's been going to the bathroom a lot. Her urine test came back positive for infection, and nothing in her medicines has changed.
6. Nurse: When I ask her to say the days of the week backward, she loses track after two and starts talking about the bus. She can't keep her attention on me for a full minute.

What a skilled clinician weighs after each clue

1. Major neurocognitive disorder, Delirium, Major depressive disorder
2. Major neurocognitive disorder, Delirium, Neurocognitive disorder with Lewy bodies
3. Delirium, Major neurocognitive disorder
4. Delirium, Major neurocognitive disorder
5. Delirium, Major neurocognitive disorder
6. Delirium

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium; Inouye, Westendorp & Saczynski (2014), Delirium in elderly people, The Lancet 383:911-922

Patient 39: The Sunday dinners

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Gloria, 77, a retired real estate agent and widow, seen at home by a county aging-services social worker. Her daughter asked for the visit and is there.

> *"My daughter worries too much. I'm perfectly fine. Would you like some coffee?"*

If asked about…

When did the changes start?	Daughter: About two years ago, slowly. First repeating questions, then getting lost driving. It's been getting worse little by little, never all at once.
Mood	I'm not sad. Why would I be sad? I have a lovely home. (Daughter: She doesn't cry or say she's down. She just doesn't start anything anymore.)
Memory	My memory is fine for my age. People move things around on me. (She tries to answer every question and fills gaps with explanations.)
Sleep and appetite	I sleep like a baby. (Daughter: She sleeps fine. She forgets lunch unless I leave a plate in the fridge.)
Medicines, alcohol and drugs	A blood pressure pill and the new one for my mood. A glass of wine at Christmas, that's all.
Medical history	Daughter: High blood pressure. Thyroid and B12 were normal, and the scan showed no strokes. The memory test was well below normal.
Daily functioning	Daughter: I've done her bills and her pillbox for about six months. She still bathes and dresses on her own. We took the car keys after she got lost.
Safety	Daughter: She left the stove on twice. (Gloria: I don't want to die, honey, I want to see my grandbabies grow up. I'd never hurt anyone.)
Loss and grief	My husband passed six years ago. I missed him terribly that first year, and then I got back to my life. I still talk to his picture sometimes.
Family history	Daughter: Her mother got very forgetful in her eighties. Nobody had depression that we know of.
Culture and context	Daughter: My brothers think Mom just needs to get out more. In our family nobody wants to say the word dementia.

Only if asked (don’t volunteer)

- Gradual decline over about two years, memory first
- Got lost driving to a familiar store
- Daughter took over bills and medications
- Stove left on twice

Patient 39: answer and debrief

DIAGNOSIS

Major neurocognitive disorder

Withdrawal and flatness first look like depression, and an antidepressant was started. Clue 3 flips it: she denies sadness and covers her lapses with explanations instead of stressing them. Clue 4 shows a slow two-year decline with memory first, and clue 5 shows she needs help with bills and medicines, so it is major, not mild. A normal workup, no strokes and a steady decline with early memory loss fit Alzheimer's disease as the likely cause. Exam lesson: depression with memory complaints has a datable onset, 'I don't know' answers and stresses the deficits; neurocognitive disorder creeps in and the person minimizes. Apathy is common early in Alzheimer's and is often mistaken for depression. Rule out reversible causes first, then address safety (driving, the stove), caregiver support and planning.

What had to be ruled out

- Major depressive disorder: denies sadness, enjoys her grandchildren, no response to an antidepressant; minimizes rather than stresses her deficits
- Persistent depressive disorder: no depressed mood most days
- Mild neurocognitive disorder: she needs help with bills and medications, so independence is lost
- Delirium and reversible causes: slow two-year course; normal thyroid and B12; no strokes on the scan

Checkpoint: What would you ask or check NEXT?

A. Ask how her memory and daily tasks have changed over time, and arrange a cognitive screen and medical workup **(best)**

B. Suggest her doctor raise the antidepressant dose, since four months has not been enough

C. Encourage her to rejoin book club as behavioral activation for depression

She denies sadness and explains away her lapses, and the antidepressant has not helped, so the diagnosis itself needs checking. A history of the course, a cognitive screen and a check for reversible causes come first. B and C keep treating a depression that has not been confirmed.

The case in six clues (as the app reveals it)

1. Daughter: Mom stopped going to book club and stopped cooking Sunday dinner. She sits in front of the TV most of the day.
2. Daughter: Her doctor started an antidepressant four months ago because she seemed so flat, and nothing is better.
3. I'm not sad, I'm fine. The book club picks books too fast, and somebody mixed up the cards in my recipe box.
4. Daughter: Looking back, it's been sliding for about two years. First she asked the same questions over and over, then she got lost driving to the grocery store she's used for twenty years.
5. Daughter: For about six months I've paid her bills and filled her pillbox, because she paid the gas bill three times and missed her blood pressure pills for a week. She still bathes and dresses herself.
6. Daughter: Her doctor checked her thyroid and B12, her brain scan showed no strokes, and her memory test was well below normal for her age and schooling. She lights up and laughs when the grandkids visit.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Major neurocognitive disorder, Persistent depressive disorder
2. Major depressive disorder, Major neurocognitive disorder, Persistent depressive disorder
3. Major neurocognitive disorder, Major depressive disorder, Mild neurocognitive disorder
4. Major neurocognitive disorder, Mild neurocognitive disorder, Major depressive disorder
5. Major neurocognitive disorder, Major depressive disorder
6. Major neurocognitive disorder

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Neurocognitive Disorder Due to Alzheimer's Disease

Patient 40: The church treasurer

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Kofi, 70, a retired civil engineer who still serves as his church's treasurer, at a memory clinic. His wife, Abena, is with him.

> *"My wife says I should get checked. I think she's right, and that frightens me a little."*

If asked about…

When did this start?	About eighteen months ago. Slowly. I noticed the church books were taking longer and longer.
What do you forget?	Names, why I came into a room, what the pastor asked me to do on Sunday. If I write it down, I'm fine.
Daily tasks	I pay our bills online, fill my own pillbox and take my pills with a phone alarm. I drive to church and the store. No accidents, and I've never gotten lost.
Sleep	I sleep fine except when I lie awake worrying about my memory. Abena says I don't snore or stop breathing.
Mood	I'm worried, not sad. I still enjoy the choir and my grandsons. I haven't lost interest in anything.
Medicines, alcohol and drugs	A blood pressure pill. The doctor stopped an allergy pill I took at night for years. I don't drink at all, and never have.
Medical history	High blood pressure, well controlled. No strokes, no head injuries. Thyroid and B12 were normal.
Testing	Abena: The doctor here said the scores showed a real drop, mostly in memory, but not a big one. He did fine on the parts about planning.
Thoughts of suicide or harming others	No. I have too much to live for. I would never harm anyone.
Family history	My older brother had dementia in his late seventies. That's why this scares me.
Culture and context	Where I grew up, an elder who forgets is talked about. I don't want the congregation to know until I understand what this is.

Only if asked (don’t volunteer)

- Testing showed a measurable drop compared with peers
- Uses alarms, a pillbox and a notebook to stay independent
- Older brother had dementia

Patient 40: answer and debrief

DIAGNOSIS

Mild neurocognitive disorder

Forgetting names and worrying about it first fit normal aging or anxiety. Clue 4 flips it: testing shows a measurable decline compared with his peers, which normal aging does not. Clue 5 keeps it mild: he still handles bills, medicines and driving, with alarms, a pillbox and a notebook. Exam lesson: mild versus major turns on independence. Extra effort, strategies or reminders mean mild; needing others to take over complex tasks such as bills or medicines means major. Rule out reversible causes such as medications, thyroid, B12 and sleep problems, then monitor over time and support planning while he can still make his own decisions.

What had to be ruled out

- No mental disorder (normal aging): testing shows a measurable decline compared with peers
- Major neurocognitive disorder: still independent with bills, medicines and driving, using strategies
- Major depressive disorder and generalized anxiety disorder: mood and interest intact; worry is about the memory change and began with it
- Medication or medical cause: thyroid and B12 normal; no change three months after stopping the allergy pill; no signs of a sleep disorder

Checkpoint: What would you ask or check NEXT?

A. Reassure him that forgetting names is normal at seventy

B. Refer him for anxiety treatment, since worry is keeping him awake

C. Ask in detail how he manages bills, medicines and driving, and arrange standardized cognitive testing and a medication review **(best)**

A clear change from his own baseline needs objective testing, a look at reversible causes such as medicines, and a functional history to judge severity. A dismisses a real decline without testing. B treats the worry, which follows the memory change rather than causing it.

The case in six clues (as the app reveals it)

1. I forget names I've known for years, and I walk into rooms and can't remember why. I lie awake worrying about it.
2. Abena: He used to do the church books in one evening. Now it takes him the whole weekend, and he double-checks everything.
3. I'm no more down than anyone my age. I still sing in the choir and play with my grandsons; the worry is about my memory, and only since this started about eighteen months ago.
4. Abena: His doctor said everyone forgets names at seventy. But the testing here showed a real drop in memory compared with men his age and schooling.
5. I still pay our bills, take my own pills and drive us to church. I just need phone alarms, a pillbox and a notebook I check every morning.
6. Abena: His doctor checked his thyroid and B12 and stopped an old allergy pill that can cloud thinking, and nothing changed after three months. He sleeps well and doesn't snore.

What a skilled clinician weighs after each clue

1. No mental disorder (normal or expected reaction), Mild neurocognitive disorder, Generalized anxiety disorder, Major depressive disorder
2. Mild neurocognitive disorder, No mental disorder (normal or expected reaction), Major neurocognitive disorder, Major depressive disorder
3. Mild neurocognitive disorder, No mental disorder (normal or expected reaction), Major neurocognitive disorder
4. Mild neurocognitive disorder, Major neurocognitive disorder
5. Mild neurocognitive disorder
6. Mild neurocognitive disorder

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major and Mild Neurocognitive Disorders

Patient 41: The lifting injury

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Shannon, 32, a home health aide, at her primary care clinic for back pain. Her doctor sits in for the first part of the visit.

> *"I just need a refill that works. I'm not some addict, okay? I work hard."*

If asked about…

The injury and pain now	I hurt my back lifting a patient about ten months ago. It still aches, but honestly the pills are more about not feeling sick now.
How many pills	Four or five a day. I tell myself two, and it never stays at two. Two used to be plenty.
Where the pills come from	The prescription ran out five months ago. My cousin's friend sells them. One text and he drops them off.
Trying to stop	Twice in the past month. I lasted a day each time. I ached, my nose ran, I had the runs and goosebumps, and I couldn't sleep.
Craving	I think about them all morning until I take one.
Work, family and driving	I've never missed a shift. I go to all my daughter's softball games. I don't drive after I take them. My boyfriend knows and he's not mad, just worried.
Other substances	A beer at a cookout once in a while. No sleeping pills or nerve pills. Nothing else.
Overdose history and naloxone	I've never passed out from them. I don't have naloxone. I usually take them alone in my bathroom so my daughter doesn't see.
Medical history	Healthy except the back. No liver problems, no hospital stays.
Thoughts of suicide or harming others	No. My daughter needs me. I'd never hurt anyone.
Family history	My uncle died of an overdose a few years ago. My mom drinks too much.
Culture and context	Where I live, everybody knows somebody who sells. People call you a junkie if you go to the clinic. That's why I came here instead.

Only if asked (don’t volunteer)

- Prescription ended five months ago; buys pills from a cousin's friend
- Two failed attempts to quit in the past month
- Usually uses alone
- Uncle died of an overdose

Patient 41: answer and debrief

DIAGNOSIS

Opioid use disorder

Pain, poor sleep and withdrawal symptoms first look like physical dependence on a prescribed medicine, which is not a disorder. Clue 3 flips it: the prescription ended five months ago and she is buying pills. Once use is no longer as prescribed, tolerance and withdrawal count. She meets 5 criteria (more than intended, failed attempts to quit, craving, tolerance, withdrawal), and clues 5 and 6 rule out the rest, so the severity is moderate (4 to 5); mild is 2 to 3 and severe is 6 or more. Exam lesson: offer harm reduction right away. Pills not from a pharmacy may contain fentanyl, so she should carry naloxone, never use alone, and know the overdose signs (pinpoint pupils, unresponsiveness, slow breathing; call 911 and give naloxone). Offer medication for OUD, such as buprenorphine or methadone, which lowers overdose risk.

What had to be ruled out

- No mental disorder: tolerance and withdrawal do not count when an opioid is taken as prescribed, but her prescription ended five months ago
- Opioid withdrawal: describes withdrawal when late on pills, but she is not in withdrawal today; it is part of the use pattern
- Somatic symptom disorder: her pain followed a real injury, and her focus is on getting pills, not on fear about her health
- Major depressive disorder: crankiness and poor sleep are tied to pain and pill timing, with no lasting low mood

Checkpoint: What would you ask or check NEXT?

A. Tell her you cannot keep treating her until she stops using pills that are not prescribed

B. Ask whether she ever uses alone or with alcohol or sleeping pills, explain that street pills may contain fentanyl, and offer naloxone **(best)**

C. Advise her to stop all opioids today and prove she can stay off them before any treatment

Pills bought outside a pharmacy may be counterfeit and contain fentanyl, so overdose risk is the first concern. Asking about the riskiest patterns and offering naloxone meets her where she is and keeps her safe while the assessment continues. A ends care and pushes her away. C is not a treatment plan, and stopping and then relapsing raises overdose risk because tolerance drops.

The case in six clues (as the app reveals it)

1. My back hasn't been right since I hurt it lifting a patient about ten months ago. I can't sleep, I'm cranky, and I need something that actually works.
2. When I'm late on my pills I ache all over, my nose runs, I get goosebumps and the runs. The two pills that used to work barely touch it now, so I take four or five.
3. Doctor: Her prescription after the injury ended five months ago, and the state prescription database shows none since. She has been getting pills somewhere else.
4. My cousin's friend sells them. I tell myself two a day and it never stays at two, and twice in the past month I tried to quit and lasted a day each time.
5. I think about them all morning until I get one in me. But I've never missed a shift or my daughter's games, and I never drive after taking them.
6. Doctor: Her boyfriend knows and isn't upset with her. Getting the pills takes one text, and she has had no health problems from them so far.

What a skilled clinician weighs after each clue

1. Somatic symptom disorder, Major depressive disorder, Opioid use disorder, Insomnia disorder
2. Opioid withdrawal, Opioid use disorder, No mental disorder (normal or expected reaction)
3. Opioid use disorder, Opioid withdrawal
4. Opioid use disorder, Somatic symptom disorder
5. Opioid use disorder
6. Opioid use disorder

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Opioid Use Disorder; SAMHSA TIP 63, Medications for Opioid Use Disorder (2021), Overdose risk and naloxone

Patient 42: The 2 a.m. business plan

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Connor, 23, an apprentice electrician, in the emergency department at 2 a.m. His roommate brought him, and the ER nurse joins the social worker.

> *"I gotta get out of here, I have so much to do. But my heart, man. Is this normal?"*

If asked about…

When did this start?	A few hours ago, at the concert. I felt amazing, then my heart started pounding and my chest got tight.
Did you take anything?	Nothing much. (If pressed:) Okay, a guy gave me two pills. He said they were Adderall. I took them around nine.
Past use	First time with pills like that. I drink a few beers on weekends. I tried weed once in high school.
Your plans	I'm going to start three companies. Solar, apps and a food truck. I'll be a millionaire by spring. (Later he says it was the pills talking.)
Sleep and past mood	I sleep seven hours most nights. I've never had a week of feeling like this or of being really down.
Medical history and medicines	Healthy. No heart problems. No prescriptions at all.
Chest pain	It's tight, right in the middle. It scares me. It's better than an hour ago.
Thoughts of suicide or harming others	No, man, I've got plans. I'm not going to hurt anyone. (When the crash comes later: I feel like garbage, but I don't want to die.)
Family history	My dad has high blood pressure. Nobody has bipolar or anything like that that I know of.
Work and daily life	I'm in my second year as an apprentice electrician. I'm on a job site at seven this morning.
Culture and context	The guys at the union hall would never let me live this down. My mom is going to kill me.

Only if asked (don’t volunteer)

- Took two pills sold as Adderall at about 9 p.m.
- First time using pills like that
- No prior mood episodes

Patient 42: answer and debrief

DIAGNOSIS

Stimulant intoxication

A racing heart and fear look like panic, and nonstop talk with grand plans looks like mania. Clue 4 flips it: this began hours ago in a man with no history of mood episodes, while mania needs a week (or hospitalization) and panic peaks within minutes. Pills sold as Adderall, a positive screen, and symptoms that cleared as the drug wore off confirm stimulant intoxication. Exam lesson: check for a substance first, and treat chest pain and high blood pressure as a medical issue. The crash afterward can bring low mood, so check suicide risk before discharge. Street pills may contain methamphetamine or fentanyl, so offer harm reduction education. One use is not a use disorder; screen his use pattern.

What had to be ruled out

- Bipolar I disorder: onset hours ago, no prior episodes, and it cleared as the drug wore off
- Substance/medication-induced bipolar disorder: the symptoms did not outlast the expected course of intoxication
- Panic disorder: symptoms lasted hours with grandiosity and positive screen; no past attacks
- Stimulant use disorder: no pattern of use shown; first time with these pills

Checkpoint: What would you ask or check NEXT?

A. Suggest a mood stabilizer for what looks like a manic episode

B. Coach slow breathing for a panic attack and plan discharge once he calms

C. Ask him and his roommate what he took tonight, and make sure the doctor checks his heart and chest pain **(best)**

A fast pulse, high blood pressure, wide pupils and chest pain with sudden grandiosity point to a substance and a possible cardiac emergency, so find the substance and get a medical check first. A labels a mood episode that began only hours ago. B treats the chest pain as panic without ruling out a medical cause.

The case in six clues (as the app reveals it)

1. My heart is going a mile a minute and I can't catch my breath. Something is really wrong with me.
2. Roommate: An hour ago he was telling everyone he's going to start three companies and be a millionaire by spring. He won't stop talking or sit down.
3. Nurse: Pulse 132, blood pressure high, pupils wide, and he's sweating. He says his chest feels tight.
4. Roommate: He was totally normal at dinner tonight. He's never had anything like this, and he's never gone days without sleep.
5. Roommate: At the concert a guy gave him two pills he said were Adderall. Connor doesn't have a prescription for anything.
6. Nurse: His urine screen is positive for amphetamine, and his heart tracing shows a fast but normal rhythm. By late morning he was sleepy, starving and embarrassed, and the big plans were gone.

What a skilled clinician weighs after each clue

1. Panic disorder, Stimulant intoxication, Anxiety disorder due to another medical condition
2. Bipolar I disorder, Stimulant intoxication, Substance/medication-induced bipolar disorder
3. Stimulant intoxication, Bipolar I disorder, Panic disorder
4. Stimulant intoxication, Substance/medication-induced bipolar disorder, Bipolar I disorder
5. Stimulant intoxication, Substance/medication-induced bipolar disorder
6. Stimulant intoxication

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Stimulant Intoxication

Patient 43: The paycheck advance

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Renata, 52, a hospital billing specialist, in a telehealth session through her employee assistance program. Her husband joins for the second half.

> *"My EAP counselor said this would be confidential. Is it? Because my job can't find out."*

If asked about…

When did this start?	About two years ago, small bets on football with an app. In the past year it got out of hand.
How much you bet	Hundreds a night now. Small bets don't feel like anything anymore.
Losses and chasing	When I lose, I go right back the next night to win it back. I'm down about thirty thousand, I think. I'm scared to add it up.
Trying to stop	Twice in the past six months I promised my husband. I deleted the apps and put them back within a week. When I stop, I'm on edge and snapping at everyone.
Sleep and mood	I'm up late betting, then tired. I've never had days of feeling great on no sleep. I feel low after a big loss, then I'm okay.
Alcohol, drugs, medicines	A glass of wine on holidays. No drugs. A cholesterol pill.
Medical history	High cholesterol. Nothing else. My doctor says I'm healthy.
Thoughts of suicide	The night he found the card, I thought everyone would be better off without me. I didn't plan anything, and I'd never do that to my kids. I'm not feeling that now. My kids and my faith keep me here.
Money and family	Husband: She borrowed rent from her sister and asked her boss for an advance. I had no idea about the second card.
Family history	My father played the horses. We never talked about it.
Work	I'm good at my job. I'm terrified my boss will figure out why I needed the advance.
Culture and context	In my family, money problems are private. You don't ask for help outside the family. Talking to you is hard.

Only if asked (don’t volunteer)

- Passive thoughts that her family would be better off without her on the night the card was found
- About thirty thousand dollars lost
- Borrowed rent money from her sister

Patient 43: answer and debrief

DIAGNOSIS

Gambling disorder

Poor sleep, guilt and late nights feeling 'electric' first suggest depression or a bipolar disorder. Clue 3 names the betting, and clue 4 flips away from bipolar: the high comes only while betting, with no days of high energy on little sleep. Chasing losses, bigger bets, lying, failed attempts to stop, irritability when cutting back, betting to escape bad moods and borrowing money meet well over the 4 of 9 criteria needed within 12 months. Exam lesson: if gambling happens only during manic episodes, do not add gambling disorder. Always assess suicide risk, since it is elevated in this disorder. It is the only behavioral addiction in DSM-5-TR Section II.

What had to be ruled out

- Bipolar I and II disorders: the excitement comes only while betting; no episodes of high energy with less need for sleep
- Major depressive disorder: mood is steady except right after losses
- Generalized anxiety disorder: worry centers on debts and secrets from gambling
- Alcohol use disorder: a glass of wine on holidays only

Checkpoint: What would you ask or check NEXT?

A. Ask about thoughts of suicide, now and in the past, and how much debt she is facing **(best)**

B. Refer her to a debt counselor before going further

C. Advise her husband to take over all the household money starting today

Problem gambling carries high rates of suicidal thoughts and attempts, especially when debts and secrets come out, so safety comes first. B is useful later but skips the risk check. C is a possible step in a plan she agrees to, not a first move made for her.

The case in six clues (as the app reveals it)

1. I'm not sleeping, I'm behind on everything, and I feel like I'm lying to everyone I love.
2. Some nights I'm up until three on my phone, and I feel electric, like I can't lose.
3. It's the betting apps. It started as a few dollars on football two years ago; now I bet hundreds, and when I lose, I bet more the next night to win it back.
4. The electric feeling only comes while I'm betting, and the next day I'm wiped out. Otherwise my mood is steady unless I've just lost big.
5. Husband: I found a second credit card she'd hidden. She has sworn twice in the past six months that she'd stop, and she asked her sister for rent money.
6. When I try to stop, I'm on edge and snapping at everyone. After a bad day I bet to feel better, and last month I asked my boss for a paycheck advance.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Generalized anxiety disorder, Gambling disorder, Alcohol use disorder
2. Bipolar II disorder, Bipolar I disorder, Gambling disorder
3. Gambling disorder, Bipolar II disorder
4. Gambling disorder, Major depressive disorder
5. Gambling disorder
6. Gambling disorder

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Gambling Disorder

Patient 44: The back steps

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Vernon, 71, a retired machinist, at a fall-prevention clinic after his second fall in four months. His daughter drove him and joins the visit.

> *"My daughter thinks I'm falling apart. I tripped on the steps. It happens."*

If asked about…

When did the tiredness start?	After I retired two years ago. I had nothing to get up for. It's been worse this past year.
Sleep	I fall asleep in my chair, then wake up at two or three and can't get back to sleep.
Mood	Not sad. Bored. I still love my woodshop, and I light up when the grandkids come.
Drinking	Three or four beers in the evening. It's just beer. I mean to have one and end up on my fourth.
The falls	Both after supper, on the back steps. The doctor said the beer plus my blood pressure pills make me dizzy. I didn't stop, I guess.
Morning drinking or shakes	Never. I don't touch it before supper. I'm in the woodshop in the mornings, clear-headed.
Driving	I don't drive after I've had a beer. My wife drives in the evenings.
Medicines and other drugs	Two blood pressure pills and a cholesterol pill. No sleeping pills. Nothing else.
Medical history	High blood pressure. A bruised hip from the second fall. My memory is fine.
Thoughts of suicide or harming others	No. I'm bored, not done. I keep my hunting rifle locked in the safe, and I haven't been out in years. (Daughter: I'd feel better if it were out of the house.)
Family history	My father drank whiskey every night. He was a mean drunk. I'm not like that.
Culture and context	At the plant, a beer after the shift was how you were one of the guys. My generation doesn't talk to counselors.

Only if asked (don’t volunteer)

- Three or four beers every night
- Doctor linked the falls to beer and blood pressure pills
- Locked hunting rifle at home

Patient 44: answer and debrief

DIAGNOSIS

Alcohol use disorder

Fatigue, poor sleep and irritability after retirement suggest depression or a normal adjustment, and clue 3 makes it tempting to call it no disorder. Clue 4 flips it: nightly drinking, and both falls came after supper. He drinks more than he means to and keeps drinking after his doctor linked it to his falls. That is at least 2 criteria within 12 months, and clue 6 rules out withdrawal and drinking before driving, so it is alcohol use disorder, mild (2 to 3). Exam lesson: in older adults, ask about alcohol when you see falls, poor sleep or fatigue. Aging bodies feel alcohol more strongly, and it interacts with common medicines such as blood pressure pills. Do not assume depression or normal aging. Ask about firearms too: heavy drinking and a gun at home raise suicide risk in older men, so discuss safe storage or removal even when he denies suicidal thoughts.

What had to be ruled out

- Major depressive disorder: low depression score; interest and enjoyment intact
- Substance/medication-induced depressive disorder: no depressed mood
- Adjustment disorder: retirement was two years ago, and the problems track the drinking
- No mental disorder: drinks more than intended and keeps drinking despite falls his doctor tied to it (2 criteria)
- Mild neurocognitive disorder: no memory or thinking complaints

Checkpoint: What would you ask or check NEXT?

A. Reassure them that tiredness and boredom are normal after retirement

B. Ask how much and how often he drinks, and when, using a screen made for older adults **(best)**

C. Suggest his doctor try an antidepressant for his low energy

Falls, poor sleep, fatigue and irritability in an older adult with a low depression score call for an alcohol and medication history. Screens suited to older adults, such as the AUDIT-C or the SMAST-G, help here. A dismisses real warning signs. C treats a depression his screen does not support.

The case in six clues (as the app reveals it)

1. I've been tired and kind of flat since I retired two years ago. I don't sleep through the night, and I've fallen twice in four months.
2. Daughter: He naps half the afternoon and gets short with Mom over little things.
3. I'm not sad, I'm bored. The doctor's depression questionnaire came out low, and I still look forward to my woodshop and my grandkids' visits.
4. Daughter: Both falls were after supper, on the back steps. These days he has three or four beers every night, and he says that's nothing.
5. I tell myself one with the ballgame, and I'm on my fourth by the ninth inning. The doctor told me the beer and my blood pressure pills are why I'm falling, and I've kept on anyway.
6. Daughter: He only drinks at home in the evening, never drives after, and has never had the shakes in the morning. It crept up so slowly that none of us noticed.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Insomnia disorder, Alcohol use disorder
2. Major depressive disorder, Adjustment disorder, Mild neurocognitive disorder
3. No mental disorder (normal or expected reaction), Adjustment disorder, Alcohol use disorder
4. Alcohol use disorder, No mental disorder (normal or expected reaction), Substance/medication-induced depressive disorder
5. Alcohol use disorder
6. Alcohol use disorder

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Alcohol Use Disorder; SAMHSA TIP 26, Treating Substance Use Disorder in Older Adults (2020), Screening and assessment

Patient 45: The court order

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Travis, 34, out of work, at a community clinic for a court-ordered evaluation after an assault charge. The clinic has his probation officer's report, and his mother joins by phone.

> *"Let's get this over with. What do I have to say so you sign the paper?"*

If asked about…

The assault	Guy at a gas station looked at me wrong, so I hit him. He had it coming. I don't feel bad. I feel bad I got caught.
Other arrests	Car theft at 19, some checks, a couple of fights. Everybody's got a past.
Work and money	Seven jobs in five years. Bosses are idiots. I owe child support, but she doesn't need it; her new guy has money.
Lying	I say what works. I told my ex I had cancer so she'd lend me money. She bought it.
Childhood and school	I was a handful. Stole stuff, skipped school, got suspended for fighting in eighth grade. (Mother: It started before he was 12.)
Alcohol and drugs	A couple of beers now and then. No drugs. My probation tests are clean.
Mood and sleep	My mood's fine. I sleep fine. I've never been up for days or felt like a superhero.
Medical history	No head injuries, no seizures. Healthy.
Anger now and violence risk	I'm not planning to hurt anybody. I'm not mad at anyone in particular. I don't own a gun; I'm on probation. If someone gets in my face, that's on them.
Feelings about people he has hurt	Not really my problem. People should look out for themselves.
Family history	My dad did time for robbery. I barely knew him.
Culture and context	Where I grew up you had to be tough or you got eaten. (Mother: His brother grew up in the same house and never got in trouble.)

Only if asked (don’t volunteer)

- Told his ex he had cancer to get money
- Stealing and cruelty to an animal before age 12
- Feels no remorse about the assault

Patient 45: answer and debrief

DIAGNOSIS

Antisocial personality disorder

A temper and a court order first suggest intermittent explosive disorder. Clues 2 and 3 flip it: theft and fraud planned for gain, plus no remorse, are not explosive outbursts followed by regret. Lying for money, irresponsibility and aggression point to antisocial rather than narcissistic personality disorder, since he seeks gain, not admiration. The diagnosis needs age 18 or older and conduct disorder that began before 15, and clue 6 supplies both. Exam lesson: never diagnose antisocial personality disorder before 18; get collateral history; assess violence risk (past violence is the best predictor) at every contact; and watch your own reactions, which are common with Cluster B clients.

What had to be ruled out

- Intermittent explosive disorder: his harm is often planned and for gain, with no remorse afterward
- Narcissistic personality disorder: seeks money and advantage, not admiration
- Borderline personality disorder: no self-harm, emptiness or fear of abandonment
- Bipolar I disorder and substance use: no mood episodes; clean drug tests and little drinking
- Conduct disorder: he is over 18 and meets criteria for antisocial personality disorder

Checkpoint: What would you ask or check NEXT?

A. Tell him his lack of remorse shows antisocial personality disorder

B. Ask about anger toward anyone now, any threats or plans, access to weapons, and his behavior since childhood **(best)**

C. Start anger-management skills training right away

A history of assault calls for a violence risk check first: current anger at a specific person, threats, plans and weapons. A full history from childhood, with collateral, is needed before any personality diagnosis. A labels him too early and adds nothing to safety. C treats the temper before the risk and the pattern are assessed.

The case in six clues (as the app reveals it)

1. I'm here because the judge said so. I've got a temper, and people push me.
2. Probation report: Since age 19 he has been arrested for assault, car theft and check fraud. He has had seven jobs in five years and has never paid the child support he owes.
3. The guy I hit had it coming; he looked at me wrong. The checks? The store has insurance, and I'd do it again if I needed the money.
4. When I want something, I say what I have to. I told my ex I had cancer so she'd lend me two thousand dollars.
5. I don't drink much and I don't do drugs; my last three probation tests were clean. I've never had days of no sleep or feeling on top of the world.
6. Mother: Before he was 12 he was stealing from my purse and from stores, and he was cruel to the neighbor's cat on purpose. By 14 he'd broken into cars and been suspended for fighting. He's 34 now, and nothing has changed.

What a skilled clinician weighs after each clue

1. Intermittent explosive disorder, Antisocial personality disorder, Borderline personality disorder
2. Antisocial personality disorder, Intermittent explosive disorder, Borderline personality disorder
3. Antisocial personality disorder, Narcissistic personality disorder, Intermittent explosive disorder
4. Antisocial personality disorder, Narcissistic personality disorder
5. Antisocial personality disorder, Narcissistic personality disorder
6. Antisocial personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Antisocial Personality Disorder

Patient 46: The orchids

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Nadia, 45, a hospital lab technician who works nights, in a telehealth visit her younger sister booked after their mother died four months ago. Her sister joins for the first part.

> *"My sister worries. I agreed to this so she would stop."*

If asked about…

Since your mother died	I miss her calls on Sundays, in a quiet way. I went to the funeral. I didn't see the point of the dinner after.
Sleep, appetite and energy	I sleep after my shift like always and eat fine. My energy is the same.
Interests	Orchids, jigsaw puzzles, true crime podcasts. I enjoy them as much as ever.
Friends and partners	I've never had a close friend or a partner, and I've never wanted one. It isn't sad to me.
Praise and criticism	The award didn't mean much. Teasing doesn't bother me. Neither one changes how I feel.
Work	I like nights because the lab is quiet. I turned down the supervisor job because I'd have to deal with people all day.
Unusual experiences	No. I don't believe in signs or messages. I've never heard voices or seen things.
Alcohol, drugs, medicines	I don't drink. No drugs. No medicines.
Medical history	Healthy. My yearly physical and thyroid tests were normal.
Thoughts of suicide or harming others	No. I'm content. I've never thought about hurting myself or anyone else.
Childhood and family history	Sister: She was a chatty little kid and played pretend with me. Around 13 she started staying in her room. Our grandfather was a very private man too.
Culture and context	My family is close. Sunday dinners at my aunt's are a big deal, and they can't understand why I'd rather be home.

Only if asked (don’t volunteer)

- Has never wanted a friend or partner
- Indifferent to praise and criticism
- Turned down a promotion to avoid supervising people

Patient 46: answer and debrief

DIAGNOSIS

Schizoid personality disorder

Withdrawal after a death suggests depression or grief, and lifelong distance from people with a turned-down promotion suggests avoidant personality disorder. Clue 4 flips it: she does not fear rejection, is unmoved by praise or criticism, and does not want closeness. No odd beliefs or perceptual experiences rule out schizotypal, and typical early social development argues against autism. Exam lesson: avoidant wants relationships but fears rejection; schizoid does not want them. Respect her goals rather than pushing socializing she has not asked for.

What had to be ruled out

- Avoidant personality disorder: no fear of rejection or criticism, and no wish for closeness
- Social anxiety disorder: not anxious in social situations; simply uninterested
- Major depressive disorder and prolonged grief disorder: sleep, appetite and interests intact; the pattern predates the loss; prolonged grief needs 12 months in adults
- Schizotypal personality disorder: no odd beliefs or unusual perceptions
- Autism spectrum disorder: typical early social communication and play

Checkpoint: What would you ask or check NEXT?

A. Ask whether she wants closer relationships, and how she feels when others judge or praise her **(best)**

B. Refer her to a social skills group to build friendships

C. Start grief therapy for prolonged grief disorder

Lifelong isolation can come from fear of rejection (avoidant) or from no wish for closeness (schizoid), and only her own view of relationships separates them. B assumes she wants friendships. C is premature: prolonged grief disorder needs at least 12 months in adults, and the pattern began long before the loss.

The case in six clues (as the app reveals it)

1. Sister: Since Mom died she's alone all the time. She doesn't call anyone, and she skipped the family dinner after the funeral.
2. I don't feel lonely. I miss Mom's phone calls in a quiet way, but I'm sleeping, eating and working like always.
3. Sister: She's been like this since she was about 13. She's never had a close friend or a partner, and she turned down a promotion because she'd have to supervise people.
4. People don't scare me; they just don't interest me. When my coworkers praised my work at the awards lunch it didn't mean much, and their teasing doesn't bother me either.
5. I'd rather spend my days off with my orchids and my puzzles. I've never wanted someone to share my life with, and I don't see why that's a problem.
6. Sister: She's not odd or superstitious, and she's never seen or heard things that weren't there. As a little girl she chatted, looked you in the eye and played pretend with me; she just grew quieter in her teens.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Avoidant personality disorder, Schizoid personality disorder, Social anxiety disorder
2. Schizoid personality disorder, Avoidant personality disorder, Social anxiety disorder
3. Avoidant personality disorder, Schizoid personality disorder, Autism spectrum disorder
4. Schizoid personality disorder, Schizotypal personality disorder, Autism spectrum disorder
5. Schizoid personality disorder, Schizotypal personality disorder, Autism spectrum disorder
6. Schizoid personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Schizoid Personality Disorder

Patient 47: The trivia night

Setting: Andre, 28, an inventory analyst for a grocery chain, at a community counseling center. He came alone and booked the visit online.

> *"I don't know if this is a real problem. I just don't have anyone, and I'm tired of it."*

If asked about…

When did this start?	Middle school. I stopped raising my hand, stopped sitting with people. It's been the same ever since.
Wanting relationships	I want friends. I want a girlfriend. I picture it all the time. I just know they'd figure out I'm not worth it.
How you see yourself	Boring, awkward, less than everyone else. I've felt that way as long as I can remember.
Situations you avoid	Parties, trivia nights, classes, anything new where I might look stupid. I turned down a team lead job.
Situations that are okay	Presenting numbers at work, calling vendors, ordering food. That's about the task, not about me.
Dating	I dated one woman for a few months. I never let her into my apartment or told her anything real. She said it felt like dating a wall.
Mood, sleep and appetite	I'm not depressed. I sleep fine and eat fine. I like my job and my video games.
Alcohol, drugs, medicines	A beer at home sometimes. I've never used alcohol to get through social stuff. No drugs. No medicines.
Medical history	Healthy. Nothing going on.
Thoughts of suicide or harming others	No. Lonely, not suicidal. I've never thought about hurting myself or anyone else.
Family history	My mom is shy. My dad was hard on me, always pointing out what I did wrong.
Culture and context	At church everyone has a big family and a wife by thirty. I feel like I'm failing a test everyone else passed.

Only if asked (don’t volunteer)

- The pattern began in middle school
- Never let his girlfriend into his apartment
- Performing at work is not a problem

Patient 47: answer and debrief

DIAGNOSIS

Avoidant personality disorder

Clue 2 rules out schizoid: he badly wants relationships. Anxiety about a trivia night then points to social anxiety disorder. Clue 4 flips it: the pattern dates from adolescence and runs through every relationship, including family and dating. He avoids new activities and a promotion for fear of looking foolish. Clue 5 shows that being watched or dealing with strangers is tolerable; what he avoids is closeness, because he believes he is inferior. Exam lesson: schizoid does not want relationships; avoidant wants them but fears rejection. Social anxiety disorder overlaps heavily and can co-occur, but a lifelong, pervasive pattern built on a core sense of inadequacy points to avoidant personality disorder.

What had to be ruled out

- Schizoid personality disorder: he wants friends and a partner
- Social anxiety disorder: overlaps heavily and can co-occur, but being watched and dealing with strangers are tolerable; what he avoids is closeness in every relationship, driven by a lifelong sense of being inferior, which fits avoidant personality disorder better
- Major depressive disorder: enjoys his job and games; no lasting low mood

Checkpoint: What would you ask or check NEXT?

A. Start exposure practice by planning for him to attend the next trivia night

B. Reassure him that he seems likable and encourage him to say yes next time

C. Ask how long this has gone on, how he sees himself, and whether it shows up in all his relationships **(best)**

Social avoidance can be a situational fear or a lifelong pattern rooted in feeling inadequate, and the history decides which. Duration, pervasiveness and self-image come first. A begins treatment before the assessment. B offers reassurance in place of assessment.

The case in six clues (as the app reveals it)

1. I don't really have friends. I eat lunch in my car, and on weekends I mostly stay home.
2. I hate it. I want friends and I want a girlfriend so much, but I'm sure once people get to know me they'll see I'm boring and stupid.
3. When a coworker invited me to trivia night, I said I had plans. My stomach was in knots for an hour just thinking about it.
4. It's been like this since middle school, with everyone: coworkers, my cousins, even the one girl I dated, who I never let into my apartment. I turned down a team lead job, and I won't take a cooking class because I might look stupid.
5. Giving my numbers at the Monday meeting, calling vendors or chatting with a cashier doesn't bother me much. It's letting anyone get close enough to see the real me.
6. I'm not depressed; I like my job and my games. I've just always believed I'm less than everyone else, deep down, and that's why I keep everyone at arm's length.

What a skilled clinician weighs after each clue

1. Social anxiety disorder, Avoidant personality disorder, Schizoid personality disorder, Major depressive disorder
2. Avoidant personality disorder, Social anxiety disorder, Major depressive disorder
3. Social anxiety disorder, Avoidant personality disorder
4. Avoidant personality disorder, Social anxiety disorder
5. Avoidant personality disorder, Social anxiety disorder
6. Avoidant personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Avoidant Personality Disorder

Patient 48: The year-end close

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Helen, 47, an accounting manager for a county office, at an outpatient clinic. Her husband asked her to come and joins the session.

> *"My husband thinks I need help. I think I'm the only one in my life who does things properly."*

If asked about…

When did this start?	I've always been like this. In college my roommates called me 'the auditor.'
The checking	Two or three times for anything with my name on it. It doesn't feel like a compulsion. It feels like doing my job right.
Unwanted thoughts	No scary thoughts or images. Nothing I try to push away. I'm not afraid something terrible will happen; I just can't stand sloppy work.
Work	Nobody else does the year-end close. They'd do it wrong. I missed a deadline last month because the report wasn't right yet. Two people on my team asked to transfer.
Leisure and vacations	I haven't taken a real vacation in six years. There's always something that needs doing properly.
Money	We have plenty saved, but you never know. The dryer still works if you hit it. (Husband: It doesn't.)
Home	The kitchen has a system. The house is tidy, not cluttered. I throw things out.
Mood, sleep and worry	I sleep fine. I'm not depressed. I don't worry about everything, just whether things are done right.
Alcohol, drugs, medicines	A glass of wine at Thanksgiving. No drugs. A thyroid pill, and my levels are normal.
Thoughts of suicide or harming others	No, never. I'm frustrated, not hopeless.
Family history	My father was a sergeant major. Everything in our house had one right way.
Culture and context	In my family, hard work and doing it right were how you earned respect. Resting felt like laziness.

Only if asked (don’t volunteer)

- No intrusive or unwanted thoughts
- Missed a deadline because the report wasn't perfect
- Has not taken a vacation in six years

Patient 48: answer and debrief

DIAGNOSIS

Obsessive-compulsive personality disorder

Repeated checking first suggests OCD. Clue 4 flips it: she has no intrusive, unwanted thoughts, and the checking feels right because it matches her standards (ego-syntonic). Overwork at the cost of leisure, refusing to delegate, reluctance to spend, rigid rules and perfectionism that delays her work, all since college, point to obsessive-compulsive personality disorder. Exam lesson: OCD is driven by unwanted obsessions and the person usually wants relief; OCPD is a lifelong style the person sees as correct. The two can co-occur.

What had to be ruled out

- Obsessive-compulsive disorder: no obsessions, and the checking is not done to neutralize a feared outcome
- Generalized anxiety disorder: no broad, hard-to-control worry; her focus is on standards and control
- Hoarding disorder: keeps money, not clutter; her home is fully usable

Checkpoint: What would you ask or check NEXT?

A. Ask whether she has unwanted, intrusive thoughts, and what she fears would happen if she did not check **(best)**

B. Start exposure and response prevention to reduce the checking

C. Coach her husband to stop going along with her kitchen rules

Checking can be a compulsion driven by unwanted obsessions (OCD) or part of a rigid, perfectionistic style that feels right to her (OCPD), and only her inner experience separates them. B starts an OCD treatment before OCD is established. C intervenes in the family before the assessment is done.

The case in six clues (as the app reveals it)

1. Husband: She checks everything two or three times, and if the kitchen isn't done her way, she redoes it.
2. I do check my team's reports more than once. If there's an error with my name on it, I can't let that go.
3. Husband: She hasn't taken a real vacation in six years. She won't let anyone at work help with the year-end close because they'll 'do it wrong.'
4. I don't have thoughts that scare me or that I try to push away. I check because sloppy work is wrong, and honestly, other people should check more.
5. Husband: We have plenty saved, but she won't spend money to replace the broken dryer. Everything has one right way, and she can't bend, even when it costs her.
6. I've been this way since college; my roommates called me 'the auditor.' Last month I missed a deadline because the report wasn't perfect yet, and two people on my team asked to transfer.

What a skilled clinician weighs after each clue

1. Obsessive-compulsive disorder, Obsessive-compulsive personality disorder, Generalized anxiety disorder
2. Obsessive-compulsive disorder, Obsessive-compulsive personality disorder, Generalized anxiety disorder
3. Obsessive-compulsive personality disorder, Obsessive-compulsive disorder
4. Obsessive-compulsive personality disorder, Obsessive-compulsive disorder
5. Obsessive-compulsive personality disorder
6. Obsessive-compulsive personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Obsessive-Compulsive Personality Disorder